

14 DAY WAIT – URGENT REFERRAL

ROYAL STOKE UNIVERSITY HOSPITAL

Tel 01782 671117

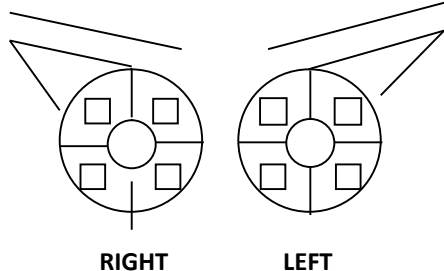
ALL REFERRALS TO BE SENT VIA RAS (REFERRAL ASSESSMENT SERVICE) YOU ARE
NO LONGER REQUIRED TO EMAIL/FAX UBRN**BREAST**

Reference: NICE Guideline recommendations for Suspected cancer: recognition and referral, July 2017

PLEASE USE THIS FORM FOR ALL PATIENTS WITH SYMPTOMATIC BREAST PROBLEMS
AND PLEASE COMPLETE ALL FIELDS

PATIENT DETAILS			GP DETAILS			
Name:	Given Name Surname		Name:	Registered GP Title Registered GP Forenames Registered GP Surname		
Address:	Home Full Address (single line)		Address:	Organisation Name, Organisation Full Address (single line)		
NHS Number:	NHS Number		Phone No:	Organisation Telephone Number		
Hospital Number:	Hospital Number		Name of referrer:			
Date of Birth:	Date of Birth		Decision to refer date:	Short date letter merged		
Sex:	☐ Male ☐ Female		Interpreter required:	☐ Yes ☐ No		
Sign Language required:	☐ Yes ☐ No		Language:	Main Language		
Contact No (next 48 hrs):	Home:	Patient Home Telephone	Work:	Patient Work Telephone	Mobile:	Patient Mobile Telephone

SUSPICION OF BREAST CANCER Please only use this section if you feel this patient is LIKELY to have Breast Cancer		SYMPTOMATIC Cancer NOT suspected	
Aged 30 or over with:		Aged less than 30 years with a lump ☐	
An unexplained breast lump with or without pain ☐		Patients with breast pain alone (no palpable abnormality) ☐ Please don't refer until primary care management has been tried as cancer is extremely unlikely (4-6 weeks regular NSAID or paracetamol as a minimum – see best practice guidance) https://www.breastcancercare.org.uk/information-support/publication/breast-pain-bcc71 <i>Referral must be accompanied by a fully completed breast pain proforma & patient to bring Breast Pain Chart completed for 3 consecutive months prior to clinic appointment</i>	
An unexplained lump in the axilla ☐		Aged less than 30 years with asymmetrical nodularity or thickening that persists at review after menstruation ☐	
Persistent breast lumpiness that does not change or resolve with the menstrual cycle ☐		Suspected abscess/infection that fails to respond to antibiotics ☐	
Blood stained or clear nipple discharge in one nipple only ☐		Any nipple discharge which is not blood stained or clear ☐	
Retraction in one nipple only ☐		Cysts in the breast tissue that keep filling up or coming back after treatment ☐	
Skin changes that may suggest breast cancer (skin tethering, dimpling, peau d' orange) ☐		With unilateral eczematous skin of areola or nipple: ☐ please don't refer until tried topical treatment such as 0.1% mometasone for 2 weeks	

Examination*Please indicate the site and size of lump*

Previous breast history (please give details including previous breast cysts and previous breast malignancy)
Please include date and location of any previous breast imaging if available

FAMILY HISTORY AND COSMETIC ISSUES

Asymptomatic patients with a family history should be referred directly by the GP to the West Midlands Family Cancer Clinical Genetics Unit by either using the family cancers questionnaire available or by calling 0121 6074757 to arrange for one to be sent to the patient.

Cosmetic breast issues (i.e. asymmetry or aesthetic issues) should not be referred to 14 day wait clinic -please refer as routine to plastic surgery

Is the patient taking an anticoagulant ☐ **Yes** ☐ **No**

If on warfarin therapy please ask patient to bring their most recent INR result to clinic

Other relevant medical history/details

Family History (please give details)

Please inform patients that because this is usually a "One Stop" service they may be in the clinic for at least 4 hours

☐ I confirm that I have discussed the possibility that the diagnosis may be cancer and they may wish to take a family member or significant other to the initial appointment.

OR

☐ I have advised the patient that they are being referred for urgent tests (symptomatic referral)

☐ I confirm that the patient is available to attend outpatients within the next 14 days

If patient is not available for the next 2 weeks, and aware of nature of referral, consider deferring the referral and seeing the patient again to reassess symptoms

ADDITIONAL INFORMATION

Allergies:
Allergies

Medication:
Medication

Relevant Medical History:
Problems